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To: [Smith, Andy E. \(DHHS\)](#)
Subject: CP - work update, 07/30/2007 - 08/24/2007
Date: Friday, August 24, 2007 5:30:02 PM

On my Plate

EPHT

Asthma/air surveillance

- revise JPHMP article
- plan next stage of analyses
- follow-up with DEP
- respond to Asthma Program's request for material for Burden Document

Water

- arsenic in private wells mapping project
- Content workgroup

Lead

- start geocoding EBLs for 2005-06, then do screeners
- assist with indicator piloting
- aggregate census tracts to towns for reporting to Portland Public Health
- moral support for database issues, and reportable #s comparison with CDC

Cancer

- develop address cleaning and geocoding protocol
- train MCR staff to manipulate, geocode, and resolve data for geocoding

Birth outcomes

- multi-state collaborative
- geocoding

Geocoding, general issues and tools

- procure data/software

Other

- meet with Tracy Kolian

Program/MeCDC

Cancer Inquiry Team

- meeting to discuss public queries, evaluate data, and respond

IPHis

- AVR Toolkit demos and evaluation

IT planning

- AD migration lists, discussions, testing
- procuring a Wiki for testing as project/program management tool

These week's activities...

EPHT

Asthma/air surveillance

The JPHMP article is DONE! (until reviewer revisions...)

Responded to DEP, never heard anything back. Andy and CP discussed and concluded that sub-area analysis not likely to illuminate anymore, based on DEP findings of region-wide ozone in 2003.

Read Asthma Burden Document developed by CSTE Fellow; MeCDC epidemiologists had requested EPHT contribution or "environmental trigger" data. The Document is a good resource as a compilation of many Maine BRFSS factoids about asthma prevalence and the characteristics of people with asthma in Maine. Overall, it's quite underwhelming though. There is no narrative about asthma in Maine! It's just endless bullet points and bar charts, with detailed tables in the appendices. The only text is an introduction which talks about asthma generally and nationwide. But nothing particularly insightful about asthma in Maine. Given all of the above, plus the fact that Katie requested only data on environmental triggers, and the fact that the case-crossover results on hand are pending publication, I didn't give them any of that material to include.

For our contribution, I gave them the updated Healthy Maine 2010 materials related to ozone. I also commented on secondhand smoke, particulate matter, and pollen, but advised that the only essential item to include was the ozone stuff from the HM2010 update.

Water

Attended 2-day conference in DC, via telephone. The first day I learned a lot about DBP, SDWIS, and methods for dealing with censored data (i.e., non-detects). The second day was digressive. Indicator packets need to be developed by 9/15. I volunteered to help wrap up the Arsenic in PWS one.

Right now there simply isn't much constituency for private well-related EPHT Indicators. SDWIS provides a rough'n'ready source applicable to much larger populations in the current crop of EPHT states. As far as indicators/data for the National EPHTN, it's probably a post-2008 goal. Need to follow up on groundwater/private well data survey.

Arsenic in private wells - geocoding and mapping

I have cleaned the addresses and filtered out a lot of chaff. This project has stalled with three impending tasks to go: 1) attempt to de-duplicate same-well results (pick first test); 2) geocode the addresses, resolving the tricky ones as possible; and 3) mapping as point plots as well as by town summary measure. That will satisfy our Year 1 goal. But it leaves much left to do, such as a) create a smoothed interpolation, b) obtain legacy data from HETL (we may already have it), c) obtain data from private testers, and d) pilot test a linkage with e.g., birth records.

Lead

Another geocoding task. EBLs should be easy, 'cause the numbers are small. But it turns out that the original address submitted with an elevated sample may be out-of-date, or otherwise not reflect the "address of exposure." Linking to the Case management system allows identifying other addresses, but which is the appropriate one to represent the child? This turns out to be tricky, and the best/only way to determine an answer is for Tina/Sue to look at the case records and figure it out. This also turned into a 12-15 hour project for MCLPPP, at a time when Sue is gone, causing some consternation.

Hopefully this will feed into an improved database design where it's easy to code and retrieve the "case-causing address".

After having MaryAnn and Kathy look it over, I reported a town-aggregated table of 2005 EBLs to Portland Public Health. Everything but Portland and Westbrook were censored.

I ran 100,000+ testee addresses through the address standardizer. Learned some things. For example, the County output can be erroneous, and the lab doesn't include State for the address field(!).

Interactions with CDC continue to confound.

Cancer

See Geocoding tools task, and Cluster Inquiry Team

Birth Outcomes

Requested that my name *not* be included as author on collaborative paper.
Referred Dan to Kathy about making data request for birth records.

Geocoding, generally

Obtained address validation software (thanks, MCR). Rhonda received invoice for Zip+4 and Zip+2 data order. One remaining tool to purchase: statewide supplemental address source.

On a regular basis it becomes clear that geocoded data is the key that unlocks a lot of analytic capabilities and makes many problems vanish. Like the problem of assigning addresses to Zip Code Tabulation Areas based on USPS zipcodes. Not a problem if the addresses are geocoded to streets. Or the problem of responding to Cluster Inquiries with area-based rates. If the data were geocoded, most of the work would be done prior to an inquiry.

Other

Met with Tracy Kolian. Clearly not a scientist, as she noted. But clearly an excellent communicator, with policy skills. Could be a good manager and risk communicator/stakeholder liason/legislative promoter. She can advocate well for EPHT, even while harboring healthy skepticism. Believes in accountability benefit of Tracking.

Program/MeCDC

Cancer Inquiry Team

Discussions surrounding North Deering (Portland), and Castine inquiries, reinforced thoughts about needing generalized, statewide analysis based on geocoded data...

IPHIS

AVR toolkits. Lots of time taken up by demos. I was left with dilemma between Business Objects as a better, more approachable data processing tool, while SAS BI is extensible to sophisticated analytics. Oracle looked to be well-supported, with the same data processing bent as Business Objects, but less approachable.

IT Planning

AD preparation. Tendered list of burning issues/questions to Jeff, who is following up swiftly. One burning issue (with ripple effect) was how the on-site application server could or could not be integrated with our S:hared directory structure. A list of hardware and software to be dealt with is nearly complete, and we have a testing machine in place. We have not run those by Jeff yet. Most of the items on your To Do list have been dealt with - either resolved, or inventoried in a list. The only item which hasn't been fleshed out is the server hierarchy and permissions, which partly depends on Jeff/OIT's solution to the question about integrating separate servers (on-site and not) into a single S:hared directory structure. The workbook of lists is here:

G:\Admin\Active_Directory\Software_Hardware_AccessMDB_list.xls

Eric and I met with Paul Sandlin (eGov services) about procuring a web-based Wiki for workgroup and project management. I was impressed with his approach and competency. To date though, he has not convinced OIT of seeding the project, so it would probably be provided with a fee, and when they get around to it. Earlier, he said the standard application fee would apply - \$250/month - but that could be shared among however many programs want to use it. He had hoped to forestall this cost for a year's worth of seeding/pilot testing. We've tagged the DWP as potential early-adopters.